

Alcohol Withdrawal — Inpatient Management

Predict severity (PAWSS), treat symptoms (CIWA-driven or front-loaded), benzodiazepines first-line · phenobarbital for severe/refractory · thiamine before glucose · don't miss DTs

1 · Predict & recognize the timeline

Risk-stratify on admission with the **PAWSS** (Prediction of Alcohol Withdrawal Severity Scale); a score ≥ 4 flags risk of complicated withdrawal. Know the clock:

Onset	Syndrome
6–24 h	Minor: tremor, anxiety, $\uparrow$ HR/BP, diaphoresis
12–48 h	Withdrawal seizures (generalized, tonic-clonic)
12–48 h	Alcoholic hallucinosis (clear sensorium)
48–96 h	Delirium tremens (confusion, autonomic storm)

Hallucinosis = hallucinations with a CLEAR sensorium and normal vitals; DTs = altered sensorium + autonomic instability. The distinction drives acuity.

2 · Treat — benzodiazepines first-line

- **Benzodiazepines reduce seizures and DTs** and are first-line (ASAM).
- **Symptom-triggered (CIWA-Ar)** dosing uses less total drug and shortens treatment vs fixed schedules — use it in patients who can be validly scored.
- **Front-loading** (large early doses of a long-acting benzo, e.g., diazepam) for high-risk/severe presentations.
- Long-acting (diazepam, chlordiazepoxide) self-tapers; use **lorazepam** with significant liver disease (no active metabolites).

CIWA-Ar is invalid in patients who can't communicate or are confused from another cause — don't drive dosing off a meaningless score; switch to fixed/front-loaded or phenobarbital protocols.

3 · Severe / refractory — phenobarbital

For severe withdrawal or **benzodiazepine-resistant** cases, **phenobarbital** is effective (alone or adjunctive) — long half-life, self-tapering, acts on GABA and glutamate.

Escalation	Note
Phenobarbital	Severe/refractory; reduces ICU/benzo needs in many protocols
Dexmedetomidine / propofol	Adjuncts in ICU — do NOT prevent seizures alone
ICU transfer	DTs, airway risk, escalating requirements

Adjuncts (clonidine, dexmedetomidine, antipsychotics) treat autonomic/agitation symptoms but do NOT prevent seizures or DTs — keep a GABAergic agent on board.

Support — vitamins & the basics

- **Thiamine BEFORE glucose** — giving dextrose first can precipitate Wernicke encephalopathy. Give parenteral thiamine generously in malnourished drinkers.
- Replete **magnesium, potassium, phosphate**; magnesium supports seizure threshold.
- Screen for and treat coexisting problems: infection, GI bleed, trauma/head injury, pancreatitis, liver disease.
- Treat the substance-use disorder: offer naltrexone/acamprosate and link to treatment at discharge.

Do/Avoid

- **DO** risk-stratify with PAWSS and pick a dosing strategy that fits the patient
- **DO** give thiamine before glucose and replete magnesium
- **DO** escalate to phenobarbital for benzo-resistant or severe withdrawal
- **AVOID** driving benzo dosing off a CIWA score in a patient who can't be validly assessed
- **AVOID** relying on clonidine/dexmedetomidine/antipsychotics alone — they don't prevent seizures
- **AVOID** under-treating: untreated DTs carry meaningful mortality